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Centers for Medicare & Medicaid Services, Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

Re: Comments on RFI CMS-0042-NC, Request for Information; Health Technology Ecosystem

Dear Center for Medicare and Medicaid Services:

Waymark, Inc. ("Waymark" or "we") is a public benefit corporation that contracts with managed care organizations and providers to provide tech-enabled community-based care to people enrolled in Medicaid programs. Waymark provides behavioral health and pharmacy services through its employed provider workforce and delivers wrap-around support to primary care providers caring for Medicaid patients using community health workers and care coordinators, reducing provider burden and addressing health-related social needs. Waymark enters into value-based contracts with Managed Medicaid Care Organizations (MCOs) using Alternative Payment Models to fund, deliver, and scale our model and align clinical outcome delivery and cost savings with our financial success. While our primary experience is in Medicaid, the principles and technology infrastructure needs we've identified are directly applicable to Medicare VBC deployment.

Waymark's care delivery model relies on using technology to improve the outcomes of Medicaid and dual eligible beneficiaries, to generate cost savings to the healthcare system, while helping scale the healthcare workforce. We commend the Center for Medicare and Medicaid Services' ("CMS") efforts to encourage, support and cultivate an ecosystem that increases data availability for stakeholders contributing to health outcomes and improves patient access to digital capabilities to inform health decisions.

Our comments and recommendations on specific areas of the RFI are set forth below.

DATA ACCESS AND INTEGRATION

PC-8c. What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?

HHS should standardize data and data sharing formats to better enable cross system data sharing across stakeholders in the system. In addition, HHS should establish secure data

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sharing protocols for VBC entities participating in federal programs, with particular focus on Medicare ACO and MA plan data integration.

There is opportunity to scale and improve the data infrastructure that is foundational to delivering better care outcomes and cost savings to the healthcare system. As a VBC provider that uses technology to identify Medicaid and dual eligible patients at risk for avoidable acute care utilization, timely and accurate data from different sources is key to finding and impacting these patients before they become high cost claimants. Below are three specific opportunities to improve accessibility, interoperability and integration of clinical data from different sources to generate actionable evidence.

1. ***Standardize ADT Feed Access***: HHS should require health systems that participate in Medicare VBC programs to provide real-time admission/discharge/transfer data to VBC entities caring for patients as this would enable timely intervention and more effective preventative care.

2. ***Enable Cross-System Data Sharing***: HHS should establish secure pathways and standardized data formats for sharing eligibility, claims, and clinical data across Medicare Advantage plans, ACOs, providers, and VBC organizations. A more comprehensive and real-time snapshot into the health status of patients would support upstream interventions in service of better clinical outcomes.

3. ***Community Resource Integration***: HHS should create standardized application programming interfaces (APIs) for connecting patients to social services, transportation, and community-based organizations that provide key social support resources to enable individuals to achieve and maintain better health. Our experience has shown that access to social supports to enable care teams to address health related social needs is particularly important for dual-eligible populations and is key to achieving better clinical outcomes and cost savings through the reduction of avoidable acute care utilization.

PROVIDERS

PR-2. What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality

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measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

Based on our experience deploying community based models of care for Medicaid and dual eligible populations, there are challenges implementing a multidisciplinary model of care that ensures evidence based prevention and reduces administrative burden that limits access. Specifically, to address those challenges, the technology infrastructure needed for VBC success and care model innovation should include 1) regulatory changes that allow and enable easier patient identification for members of a patient's care team, including trusted third party vendors, 2) regulatory clarity and support for automated scheduling and care coordination, and 3) real time data access to data needed to inform evidence based clinical and preventative care.

Enabling Patient Outreach and Engagement. Current interpretations of the HIPAA Privacy Rule create unnecessary friction in patient outreach, which does not serve the purpose of reaching patients and providing them with key information to inform decision making and health related social supports to help maintain and achieve better health. VBC entities engaging in or partnering with other entities in the care workflows need clearer guidance on when and how they can proactively contact beneficiaries without prior explicit consent, particularly for preventive interventions and care gap closure. Current TCPA regulations, even under the healthcare exemption, put up unnecessary barriers to contacting patients via text messages, the primary means of communication for many Americans.

In short, engagement of members is the first step in creating the value around disease prevention, health promotion, and reduced overall costs.

Automated Scheduling and Care Coordination: Technologies like AI-powered patient schedulers can dramatically improve care gap closure rates, but in order to encourage investment in the development and deployment of these technologies, companies would benefit from regulatory clarity around patient consent requirements, liability, if any, for use of automated technologies, and requirements for workflow integration.

With respect to the deployment of automated schedulers, appointment reminders, and other automated tools that support care coordination, it is unclear what type of patient consent should be secured and how. To provide regulatory clarity and encourage investment in the deployment and utilization of automated scheduling tools, HHS could establish the following tiered consent model.

> Tier 1 (Opt-in): Explicit written consent for AI systems to schedule appointments directly with provider offices on patient's behalf

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Tier 2 (Notification-based): Allow automated scheduling of patient appointments with 48-hour advance notification and opt-out option for established patient-provider relationships

Tier 3 (Emergency/Gap Care): Permit automated scheduling for urgent care gaps (e.g., overdue cancer screenings) with same-day notification and immediate opt-out capability

Across all of these options, patients should retain the right to revoke consent for using automated scheduling tools at any time, via a simple, and easy to follow process.

In addition, there is opportunity for HHS to incentivize and support integration with provider workflows. Specifically, HHS should mandate that EHR vendors cannot block legitimate third party scheduling integrations that meet FHIR R4 standards and patient consent requirements. Further, building upon the work of the information blocking rules, HHS should establish "must accommodate" standards for appointment scheduling APIs and clarify that sharing appointment availability and scheduling data with permitted third parties falls under the existing use and disclosures of patient information HIPAA rules that do not require explicit patient consent.

Last, for providers and entities that are willing to deploy innovative applications in provider offices, HHS can create a safe harbor where providers who accept AI scheduled appointments are protected from liability as long as the following criteria are met:

- Patient consent has been provided (see proposal above re: patient consent); and
- Scheduling systems clearly identify themselves as automated agents to provider staff
- Use of automated scheduling systems that maintain ≥95% accuracy in appointment details (time, location, provider, reason for appointment) and include a mandatory audit trail that can be monitored and reviewed for all automated scheduling activities

These digital tools will still require a human in the loop to ensure the digitally-enabled workflow is progressing as intended and high quality care.

Real-Time Data Access to Enable Deployment and Effective Utilization of Technology Tools:

Technology is impactful in driving outcomes if in its development, it is designed to be accurate, without bias and robust. HHS should play a role in facilitating access to robust, aggregated data sets, to ensure the technology developed is robust and accurate in promoting and driving evidence based prevention.

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While there has been progress in providing healthcare providers access to timely patient information to support evidence based prevention, there is still a way to go. For example, Waymark, as a VBC provider, has been able to access ADT feed data from vendors about the patients we are serving, but that data is incomplete. Patients receive care from multiple health systems, but the ADT feed will only be populated with information from healthcare entities that are contracted with that vendor. As a result, an example of the missed opportunities to address and promote health such as missing behavioral health crisis intervention opportunities that occur in emergency departments that are not contracted with an ADT vendor.

In short, success in value based care arrangements, where providers are rewarded for outcomes, requires timely and accurate data integration, that includes, at a minimum, the ability to receive:

- Same-day comprehensive ADT feed access for crisis intervention
- Real-time insurance eligibility verification to prevent care disruptions
- Streamlined prior authorization data sharing to help ensure timely care delivery

VALUE BASED CARE ORGANIZATIONS

VB-1. What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

Digital health management and care navigation products must enable, not replace, human-centered care. The failure to develop products without this reality in mind hinders the most effective deployment of digital products for the improvement of healthcare delivery and healthcare systems. HHS should mandate algorithmic bias testing for all predictive models used in federal VBC contracts to ensure that evidence-based interventions are being deployed to address the health needs of Medicare and Medicaid patients. HHS could further incentivize the use of digital health management and care navigation products by issuing guidance that clarifies how entities can comply with HIPAA requirements while engaging in patient outreach, particularly for preventive care and care gap closure activities.

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Enabling Human Centered Care, Published outcomes[1] demonstrate that technology alone is insufficient to significantly improve patient outcomes for many major chronic and behavioral health conditions. In behavioral health, the American Society of Addiction Medicine and the American Academy of Addiction Psychiatry explicitly caution against the use of standalone technology-delivered interventions for stimulant use disorder, noting insufficient evidence for their effectiveness as monotherapy and emphasizing the need for clinician involvement to monitor for decompensation and address access disparities.[2] Similarly, meta-analyses in digital mental health highlight that interventions with some degree of human support are more effective than purely self-administered, technology-only approaches.[3]

Furthermore, high attrition rates and limited patient engagement with digital tools are well-documented barriers, and the literature underscores that active patient-provider collaboration, coaching, and integration with traditional care are critical for achieving meaningful clinical benefit.[4] Thus, the consensus in the medical literature is that technology should be viewed as an adjunct to, rather than a replacement for, comprehensive care.

The combination of predictive analytics with trained community health workers, therapists, and pharmacists achieves superior results compared to purely technological interventions. VBC payment models should recognize and reimburse the essential human workforce required for

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[1] Effectiveness of Digital Health Interventions for Chronic Conditions Management in European Primary Care Settings: Systematic Review and Meta-Analysis. Ambrosi E, Mezzalira E, Canzan F, et al. International Journal of Medical Informatics. 2025;196:105820. doi:10.1016/j.ijmedinf.2025.105820. https://pubmed.ncbi.nlm.nih.gov/39922064; Randomised Controlled Trials Addressing How the Clinical Application of Information and Communication Technology Impacts the Quality of Patient Care-a Systematic Review and Meta-Analysis. Mikkonen K, Yamakawa M, Tomietto M, et al. Journal of Clinical Nursing. 2023;32(13-14):3295-3314. doi:10.1111/jocn.16448. https://pubmed.ncbi.nlm.nih.gov/35851972.

[2] The ASAM/AAAP Clinical Practice Guideline on the Management of Stimulant Use Disorder. Journal of Addiction Medicine. 2024 May-Jun 01;18(1S Suppl 1):1-56. doi:10.1097/ADM.0000000000001299. https://pubmed.ncbi.nlm.nih.gov/38669101.

[3] The Impact of Technology Systems and Level of Support in Digital Mental Health Interventions: A Secondary Meta-Analysis. Sasseville M, LeBlanc A, Tchuente J, et al. Systematic Reviews. 2023;12(1):78. doi:10.1186/s13643-023-02241-1. https://pubmed.ncbi.nlm.nih.gov/37143171.

[4] Mobile Devices and Health. Sim I. The New England Journal of Medicine. 2019;381(10):956-968. doi:10.1056/NEJMra1806949. https://www.nejm.org/doi/full/10.1056/NEJMra1806949; Randomised Controlled Trials Addressing How the Clinical Application of Information and Communication Technology Impacts the Quality of Patient Care-a Systematic Review and Meta-Analysis. Mikkonen K, Yamakawa M, Tomietto M, et al. Journal of Clinical Nursing. 2023;32(13-14):3295-3314. doi:10.1111/jocn.16448. https://pubmed.ncbi.nlm.nih.gov/35851972.

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effective technology deployment, including non-clinical workforce, such as community health workers and care coordinators who serve as bridges between technology insights and patient engagement.

An implementation model to enable use of technology more effectively with patients would include:

- Predictive algorithms to identify optimal intervention timing
- Community-based teams to provide culturally competent, trauma-informed care
- AI-powered tools to reduce administrative burden
- Real-time data integration to enable proactive outreach

Support and Incentivize Deployment of Evidence-Based Care. HHS is also well positioned to support the development and deployment of bias-aware algorithms that eliminate demographic prediction disparities by focusing on utilization patterns rather than cost alone. Requiring VBC payment models to use bias-tested algorithms and report outcomes stratified by race, ethnicity, and other demographic factors could help achieve better outcomes for all populations, especially given the VBC models and Medicare programs serve diverse populations.

VB-2. How can key themes and technologies such as artificial intelligence, population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement be better integrated into APM requirements?

HHS should make technology, data, and analytics investments to support engagement of members that are not attributed to a provider or health plan. With such investments, APM requirements that reward engagement of unattributed members, and incorporate evidence-based activities that have been shown to improve clinical outcomes would position and encourage more providers to pursue preventive care activities and drive cost savings in the system via APM. HHS should launch Medicare VBC pilots specifically designed around rising-risk identification and early intervention, building on existing ACO and MA programs to further encourage preventative and personalized activities that lead to better health outcomes and cost savings for the system.

We recommend that HHS incorporate the following items into APM requirements –

1. ***Patient Engagement.*** HHS should set an expectation in value based contracts for engaging unattributed members, and provide both investment in, and incentives to, VBC providers to do so. Value is driven by engaging members, including the underutilized opportunity to engage "rising risk" members, by providing education and encouraging

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behavior change and modification, there should be recognition and support in VBC contract structures for engagement given the critical role it plays in driving value.

2. ***Algorithm Transparency Requirements***: HHS should mandate that VBC contracts using AI/ML provide explainable predictions and bias testing results. In that way, outcomes generated would not be skewed and would reflect the work put in to improve a patient's health.
3. ***Performance Standards***: HHS should establish minimum accuracy thresholds for predictive models used in VBC payment calculations.
4. ***Continuous Monitoring***: HHS should require ongoing validation of algorithm performance across demographic groups. Without ongoing validation, it is likely providers will continue to use models that no longer meet their intended goal.
5. ***Risk-Adjusted Shared Savings***: HHS should tie VBC payments to risk-adjusted outcomes that account for patient social complexity. CMS should establish prospective baseline methodologies that account for social risk factors, ensuring that VBC entities serving more complex populations are not penalized. This could include standardized social risk adjustment similar to the Dual Eligible Special Needs Plan (D-SNP) model, with baselines set prospectively using validated social complexity measures.
6. ***Early Intervention Bonuses***: HHS could provide additional payments for successful interventions with rising-risk patients before they become high-cost, particularly relevant for Medicare populations where early intervention can prevent expensive complications. In doing so, that would encourage VBC entities to make the investments in proactive preventative healthcare measures for patients.
7. ***Technology Investment Support:*** HHS should create mechanisms for VBC entities to recover technology development costs through sustainable payment models. Specific recommendations include:

    - Multi-year contract guarantees (minimum 3-5 years) to justify upfront technology investments
    - Technology development amortization allowing VBC entities to recover AI/ML development costs over contract duration
    - Performance-based investment recovery where successful outcomes unlock additional payments to fund continued technology advancement
    - Shared risk corridors that protect VBC entities from excessive downside risk during technology implementation periods

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VB-3. What are essential health IT capabilities for value-based care arrangements?

Value-based arrangements should incorporate predictive analytics that target "rising risk" populations, capitalizing on using evidence to prevent broader disease and promote health. HHS should establish standards for data integration of social factors in VBC contracts and create pathways for sharing community resource databases across regions. Medicare ACOs and MA plans should have access to standardized social factors datasets to improve risk prediction and intervention targeting.

Traditional VBC models focus on high-cost patients after they become expensive, missing critical intervention opportunities. Better patient outcomes and cost savings could be realized by deploying machine learning models that identify "rising risk" patients before they become high utilizers. HHS should incentivize VBC contracts that reward early intervention with rising-risk populations, not just management of existing high-cost patients. This applies particularly to Medicare Advantage and ACO programs where early intervention can prevent costly acute episodes.

Most VBC models inadequately address health related social needs (HRSNs) that drive acute care utilization across all populations, including Medicare beneficiaries. HRSN data integrated into predictive models and clinical workflows would yield the following benefits –

- Area-level social determinant variables improve model performance
- Generative AI tools match patients to appropriate community resources
- Automated workflows trigger interventions based on social risk factors

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Effective tech-enabled value-based care requires thoughtful integration of predictive analytics, human-centered care delivery, and comprehensive support systems. Our experience demonstrates that technology's greatest value lies not in replacing human connections but in enabling more precise, timely, and equitable interventions.

We urge HHS to design policies that recognize VBC as a socio-technical system requiring both advanced analytics and community-based care delivery. Success depends on addressing regulatory barriers to technology deployment while maintaining focus on health equity and measurable outcomes.

Waymark stands ready to share detailed implementation guidance and collaborate on pilot programs that advance these principles within Medicare and other federal health programs.

Thank you for the opportunity to comment on the RFI.

Sincerely,

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